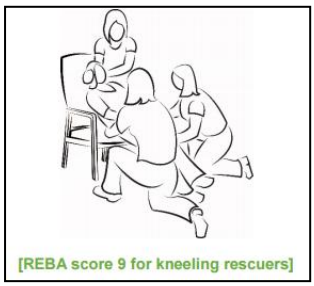
Guidance for safer handling during resuscitation in healthcare settings 2015 (Resuscitation Council UK)

Cardiorespiratory arrest in a seated patient:

To enable you to provide effective chest compressions the patient must be lowered to the floor. This should be carried out in a safe and controlled manner, ensuring that attachments (e.g. drips, drains, catheters, cannulae) are not dislodged. Transferring an unconscious patient from a seat on to the floor places the rescuer(s) at high risk of injury. The optimal number of people needed to perform this transfer is three. If fewer than three people are available, you may need to attempt a less-than-optimal transfer. Wherever possible, wait for additional people to provide assistance, but the urgency of a cardiorespiratory arrest may not allow this.

In general do not try to transfer a patient directly from a chair to a bed or trolley. A rare exception to this may occur if the patient is already sitting on a sling and a hoist is readily available. Three-person transfer from chair to floor

Three-person transfer from chair to floor:


[REBA score 9 for kneeling rescuers]

• Make sure that the chair is secure with any brakes (e.g. on a wheelchair) in the ON position.

• If a slide sheet is readily available, place it under the patient's feet and then extend their legs to slide the feet and legs away from the chair as the patient is lowered on to the floor.

• One rescuer supports the head by standing at the side of the chair level with the patient's head.

• The other two rescuers face the patient in the chair and position themselves slightly in front and to the side of the chair.

These rescuers get into a high-kneeling or half-kneeling position. With their outermost hand (furthest from the patient) they hold the patient at the back of the hip and pelvis, and place their innermost hand behind the knee. If access is restricted around the patient's hip and pelvis, they hold the patient with both hands around the back of the knee.

• On the command from one rescuer, each kneeling rescuer transfers their body weight towards their heels. The first stage is to pull the patient forward out of the chair into a sitting position on the floor with their back resting against the chair. The rescuer supporting the patient's head will need to move into a half-kneeling position during this manoeuvre in order to maintain support for the patient's head

A fourth person may assist by holding the back of the chair, to prevent it moving as the patient is moved out of the chair. This often occurs when the chair has wheels. Even with its

brakes on, a chair may slide on smooth floor surfaces. Once the patient is in a sitting position on the floor EITHER move the chair and lower the patient's head and back carefully to the floor OR pull the patient's legs forward away from the chair until the patient is lying supine on the floor. One rescuer should support the patient's head at all times. A fourth rescuer can help by moving the chair away when necessary. NOTE: A pillow placed on the floor to cushion the transfer acts as a hindrance rather than a help.

REBA score 9 – Kneeling rescuers

The patient is now seated on the floor; their upper torso must be lowered to the floor with maintained support for the head Two-person transfer from chair to floor

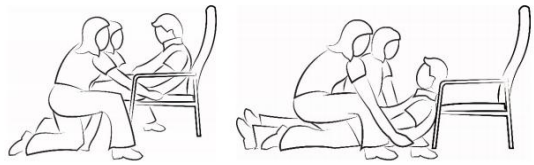
• Both rescuers face the patient in the chair, and position themselves slightly in front and to the side of the chair.

• If readily available, place a slide sheet under the patient's feet.

• Both rescuers get into a half-kneeling position with their innermost knee (nearest to the patient) on the floor and grasp hold of the patient at the back of the pelvis/hip region with their outermost hand and behind the patient's knee with their innermost hand. Alternatively, use the high-kneeling position, which some rescuers may find more comfortable.

• On the command from one rescuer, each kneeling rescuer transfers their body weight back towards their heels. This pulls the patient forwards out of the chair into a sitting position on the floor with their back resting against the chair.

Two-person transfer from chair to floor:



[REBA score 9] [REBA score 11]

CAUTION: This transfer may occur at speed because of the effect of gravity on the unconscious patient. A pillow placed on the floor to cushion the fall acts as a hindrance rather than a help.

• Once the patient is in the sitting position on the floor, one rescuer supports the patient's head, whilst the other pulls the patient's legs forwards and away from the chair or, if there is enough room, moves the chair. Alternatively, one rescuer gently pushes the patient sideways towards the other rescuer who lowers them to the floor.

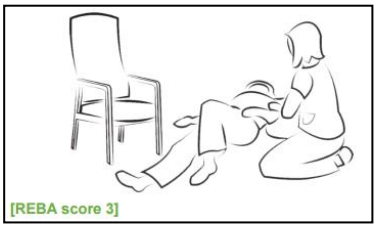
One-person transfer from chair to floor:

Wherever possible this transfer should not be undertaken by a single rescuer, who should wait for more help to arrive. However, in some situations, after calling for help, a single rescuer may wish to start immediate CPR for which the patient must be moved on to the floor. This is a high-risk manoeuvre and should be undertaken only in life-threatening or exceptional circumstances.

• Kneel on the floor to one side of the patient.

• Position the patient's arm that is closest to you across their chest and push their trunk away from you.

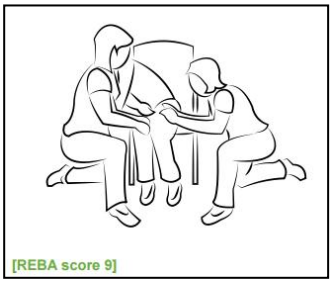
• Push against the patient's lower thigh which is nearest to you with both hands to position the patient's hips towards the front of the chair.



• Place one hand behind the patient and around their furthest hip.

• Place your other hand on the patient's thigh that is closest to you.

• Pull with the hand that is on the hip and push with the hand that is on the thigh to move the patient down to the floor.



This technique can be adapted to become a two-person transfer with the second rescuer positioned on the opposite side, pulling the patient's knees towards them with both hands. This may be particularly useful where there is limited space in front of a chair.

Cardiorespiratory arrest on a toilet

If a patient has a cardiorespiratory arrest whilst on a toilet they will be likely to fall either sideways or forwards. If the patient remains on the toilet transfer them on to the floor using one or a combination of the above techniques, depending on space constraints. Whilst transferring the patient on to the floor be sure to keep the door open. This will ensure that the entrance is not blocked and will allow other rescuers access to the room. During this transfer care should be taken to avoid trauma to the male genitalia.